This literature review argues that the mental health system in the United States is structurally inadequate for addressing race-related trauma. Racial discrimination is not only a stressor but a documented cause of post-traumatic stress disorder (PTSD). It is ongoing rather than past-tense, unevenly distributed across racial groups, and poorly served by existing measurement tools and treatment protocols. The consequences of this inadequacy are traceable through elevated rates of suicide, unemployment, and diminished quality of life in communities of color. The research reviewed here argues that these outcomes are not incidental but predictable, given a system that was not built to account for the chronic, systemic nature of racial harm.

The interplay of suicide, unemployment, race, and PTSD is dynamic and complex. The relationship between suicide and PTSD is established, as is the relationship between unemployment and suicide. Those who have PTSD may struggle to keep or attain employment, and the lack of employment may lead to suicide risk. Race-related PTSD and disparities within the mental health system based on race may exacerbate PTSD symptomology and suicide risk. Suicide risk may also be moderated or influenced by race. Neither suicide risk nor PTSD is distributed equally across race. Suicide risk and unemployment rates are different for those living with PTSD stemming from certain trauma types. Refugees and immigrants are more at risk for suicide than the general population.

The inclusion of race-related stress, experienced racism, prejudice, micro-aggression, and race-related trauma in research regarding PTSD and other trauma-related mental health sequelae is a new addition to contemporary psychological research (Carter, 2007). The Diagnostic and Statistical Manual of Mental Disorders (DSM-5; American Psychiatric Association, 2013) is used as a standard by clinicians and recognized for insurance purposes. Inclusion of race-related trauma in the current edition has been called for (Williams et al., 2018, Holmes et al., 2016).

While current debates rage on about whether racism exists and what it is (UMass Lowell, 2020; Pew Research Center, 2019), the growing body of research regarding this phenomenon is clear: racial discrimination is highly associated with PTSD and other mental health diagnoses (Bird et al., 2021, Carter 2020, Sibrava et al., 2019). Research has illustrated a narrative that living as a non-white individual in the U.S. may result in a higher chance of experiencing mental health conditions so painful that a person may end their own life.

A lack of unified understanding of the relationship between race and PTSD results in a treatment system that, at best, does not address related mental health issues and at worst, exacerbates them: one of the most prominent scales used to measure racism is called the 'perceived' racism scale (The Perceived Racism Scale, McNeilly et al., 1996). Arguments against the use of such terminology have been made (Bird et al., 2021, Cheng & Mallinckrodt, 2015). To make a blunt comparison, it would be insulting to a patient and damaging to the treatment process to ask a veteran to complete a questionnaire that labeled the trauma of what they witnessed or experienced as 'perceived trauma'.

Furthermore, currently accepted treatment methodologies for PTSD may not be effective for race-related PTSD. Exposure therapy and CBT treat PTSD by assuring patients that the trauma happened but is over, and subsequently trying to retrain the mind to live in a more peaceable present, rather than living in a state of hypervigilance or fear. For those with race-related PTSD, discrimination experienced in systemic racism may be ongoing, and these treatments are not appropriate (Bird et al., 2021). In this light, heavy research and funding should be applied to race-related mental health treatments, but that is unlikely given the lack of acknowledgement regarding the existence of race-related trauma.

More than one study has shown that ethnic identity and race-related stress moderate PTSD, meaning that ethnic identity is associated with PTSD symptomology in response to racism. Race-related stress also predicted PTSD symptomology (Khaylis et al., 2007, Polanco-Roman et al., 2015). Adverse race-related incidents have been shown to be associated with increased scores on clinician-administered scales for PTSD in Asian American Vietnam veterans (Loo et al., 2005). PTSD has long been associated with experiences of war (Crocq & Crocq, 2022). Active service members who sought treatment who were Latino or African American were reported to have re-experienced symptoms, fear, guilt and numbing more than white participants (Asnaani & Hall-Clark, 2017). In a qualitative study of Asian American veterans, Loo (1994) explores the experiences of Asian American service members in the Vietnam war. This is a singular situation in which Asian Americans are of the same race category as the native people defending themselves from American soldiers, possibly resulting in being subjected to substantially more severe acts of bias and racism. This experience of racism by Asian American service members is not limited to the intersection of identity to native Vietnamese. In situations which may lead to PTSD, especially prolonged situations of danger, soldiers need to depend on their peers for emotional support. Higher morale among the group can protect against psychological issues (Grinker & Spiegel, 1945; Whitesell, 2012). However, if a member experiences racism from within members of the group, this protective factor may be violated.

PTSD is not evenly distributed across races. PTSD has been shown to be highest among Black people (possibly due to racial trauma and witnessing domestic violence and child maltreatment), intermediate among Latinos (witnessing domestic violence and child maltreatment) and white people, and lowest among Asian Americans (Roberts, 2011). The reduced number of Asian Americans with PTSD may be due to increased mental health stigma for Asian Americans (Chu et al., 2021). One can assume that the racial differences in PTSD occurrence may be due to a variety of social, cultural, and systemic variables.

Suicide risk is associated with PTSD, both in civilians (Tarrier & Gregg, 2004) and veterans (Prigerson et al., 2001). In one study, close to 10% of civilians with PTSD made a suicide attempt since the trauma associated with PTSD occurred, with 38% reporting suicidal ideation. There are mediating psychiatric conditions that link PTSD and suicide. Those that have other mental health issues are more likely to experience suicidal ideation and attempt (Krysinska & Lester, 2010). In participants who had PTSD-related to a previous suicidal attempt, those with low levels of depression symptoms scored higher on scales measuring suicidal intent in one study (Stanley et al., 2019). When looking at PTSD by trauma type, those with PTSD who had experienced childhood maltreatment, assaultive violence, and peacekeeping (UN peacekeepers, those who are employed in post-conflict or post-disaster situations) traumas experienced the most suicidal ideation and attempt (LeBouthillier et al., 2015).

Immigrants and refugees are especially vulnerable to suicide (Amiri, 2020) with increased trauma associated with greater suicide risk (Bhui et al., 2003). As with PTSD, suicide risk is not evenly distributed between races. With the onset of COVID-19, different subsets of the population have shown increased suicide risk, notably Black and Asian American or Pacific Islander youth (Ramchand et al., 2021).

PTSD may result in lower quality of life by impacting employment status and intimate relationship health. Veterans who report combat trauma as their most traumatic event may be more likely to be unemployed (Prigerson et al., 2001) as well as divorced, fired, or physically abusive to their spouses. Additionally, PTSD is associated with sexual dysfunction, especially for those who have experienced sexual violence as the index trauma, or the trauma that marks the beginning of PTSD (O'Driscoll & Flanagan, 2016). The chronic physiological activation documented by McNeilly et al. (2002) is compounded by racial economics. Black Americans face

unemployment rates consistently twice that of white Americans (Pew Research Center, 2019). These conditions both reflect and perpetuate the structural inequities driving racial trauma.

When PTSD and unemployment compound each other, the stakes escalate. Prigerson et al. (2001) found that the severity of PTSD coupled with unemployment predicted suicidal attempt, a finding that connects the functional consequences of trauma directly to the suicide outcomes examined in this review. Unemployment is associated with greater suicide risk and attempt (Milner et al., 2013; Lundin & Hemmingsson, 2009), with those in the first five years of unemployment at the greatest risk. Kposowa (2001) found that this risk is gendered: unemployed men were twice as likely as employed men to die by suicide, while for women, risk increased with each year of unemployment. Unemployment protection in European countries has been shown to mitigate suicide risk in the context of economic recession (Norström & Grönqvist, 2015).

In conclusion, the research reviewed here points to a mental health system that has not yet reckoned with the chronic, structural nature of racial trauma. Trauma can trigger life-long mental health sequelae, and dysfunction in work, love, and the capacity to endure, much less enjoy, the highs and lows of life. When that trauma is caused by or exacerbated in ongoing racial discrimination rather than a discrete past event, the standard diagnostic, clinical, and interventional frameworks fail those most affected. Closing this gap requires not only new treatment modalities but a foundational shift in how mental health research acknowledges and measures racial harm. The field cannot effectively treat what it won’t see.

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